جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : رباح نادى ضاحى حسن

المحافظة : المنيا **الحي / المدينة :** دير مواس **العنوان :** العماره الشرقيه

رقم البطاقة : 29207272400329 **رقم التليفون :** 01125887674 **تاريخ تسجيل الإستمارة :** 18-10-2026

العمر(تاريخ الميلاد) : 27-07-1992 **المهنة :** حاصل علي دبلوم

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: رباح نادى ضاحي حسن
National ID: 29207272400329

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



National ID - Front

[[DOCTYPE:ID_FRONT]]


٢٩٢٠٧٢٧٢٤٠٠٣٢٩ ٢٠٢٦/٠٢
دبلوم تجاره
أنثى مسلمة متزوجة
هانى هاشم حسين مزيون
البطاقة سارية حتى ٢٠٣٣/٠٢/٢٠
٦

Name: رباح نادي صاحي **MR. :** 6588	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has metastatic breast cancer (MBC) with bone and liver involvement and is currently on an aromatase inhibitor (AI); the most recent PET-CT was performed to assess eligibility for CDK4/6 inhibition and Faslodex therapy.

A new biopsy of axillary lymph nodes confirmed invasive ductal carcinoma, grade 2, estrogen-receptor positive, progesterone-receptor positive, and HER2-negative by SISH. The treatment plan was initiated on cycle 1 day 1 (C1 D1) with Faslodex, Ibrance, and monthly Zometa for three months.

On cycle 1 day 15 (C1 D15) the patient continued Faslodex and Ibrance with mild clinical improvement.

Laboratory monitoring on 29 March showed total leukocyte count (TLC) 3.1 ×10⁹/L and absolute neutrophil count (ANC) 1.15 ×10⁹/L, corresponding to grade 2 hematologic toxicity; the dose was maintained, and Zometa was continued.

On 29 April the patient received Ibrance, Faslodex, and Zometa in the day-care setting after laboratory review.

The patient presented with severe back pain and underwent a PET-CT; pain management was coordinated with the pain clinic while continuing Ibrance and Faslodex (designated M4).

Chemotherapy was switched to a Taxol-based regimen combined with carboplatin; neutropenia prophylaxis with Neupogen (4 amp) was administered.

During week 6 the patient received Taxol 100 mg and carboplatin 150 mg, together with four doses of Neupogen as per Dr. Mohamed Ismail's consultation.

On 6 August the record noted a history of right-sided mastectomy, recurrent chemotherapy, and bone metastases, with ongoing Faslodex and Ibrance for four months, followed by initiation of Taxol and carboplatin for week 7 (Taxol 100 mg, carboplatin 150 mg) and one prophylactic dose of Neupogen; a PET-CT was planned, and a left axillary mass measuring 2 × 3 cm was documented.

For week 8 the patient received Taxol and carboplatin; laboratory values on 9 August showed TLC 1 ×10⁹/L, prompting administration of G-CSF and repeat CBC with additional G-CSF per Dr. Mohamed Ismail's recommendation. A subsequent CBC revealed neutrophils 0.7 ×10⁹/L, leading to two doses of G-CSF and postponement of that week's chemotherapy.

In week 9 the patient was scheduled for Taxol 100 mg and carboplatin 150 mg; a blood transfusion was planned, PET-CT and further staging were

تحريرا في ,

التاريخ :	21-09-2026	الوقت :	

الطبيب المعالج : mohamed ismael

يعتمد ,

pending, and liver function showed ALT 66 U/L. The patient has now completed week 9 of the Taxol and carboplatin regimen.

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 15/2/2026 G

Name	[illegible]	MRN	1060/2025
		ID	
Age	39yr	Phone	01112340930
Menopausal status	premenopausal	Marital status	Married
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	MBC
CO morbidity	Not DM or HTN		

Clinical Data

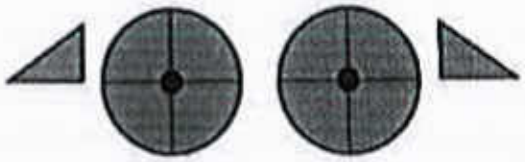
BX:

ER:	PR:	HER 2:	KI67:
+ve	+ve	-ve	

Sonomammography:

Mets Workup: PET/CT → DP of bone mets

R L



TNM

Recommendations: A Case of ♀ pt ♂ 39yo ♂ Rt Breast Ca. Rt MRM 10/2024 then imaging post operative discovered bone mets palliative. CTH + 6 Cycles AC then AI + Zoladex + Zometa last PET/CT → DP

MDT OUTCOME & PLAN for CDK 4/6 + fasledex + Zoladex + Zometa

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

GUSTAVE ROUSSY
INTERNATIONAL

٦٥٨٨

Patient Name	: الأستاذه / رباح نادي ضاحي	Request No.	: 13004821
Patient No.	: 29207272400329	Request Date	: 02/02/2026
Path No.	: 300/26	Verification Date	: 17/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 33 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Suspicious left axillary lymph node after right simple mastectomy.

Nature of specimen
Tru cut needle biopsy collected on 0 2/ 02 / 2026 , at 11 :40 a.m.
cold ischemia time is 10 seconds
Gross examination
Multiple(4) grey white tissue cores ranging in size from 0.2 cm up to 1.5 cm
Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: HER2/NEU (4B5),KI67 (30-9), PR (1E2), ER (SP1), SISH.
Microscopic examination
* Histological type: Nodal tissue cores with metastatic deposits of breast origin , focall necrotized
• IPT (Positive control showed appropriate reactivity) :
*GATA3:Diffuse positive nuclear statining.
* ER: Strong nuclear reaction in 90% tumor cells (Allred score 8/8)
* PR: Strong nuclear reaction in 90% tumor cells (Allred score 8/8)
* HER2: Moderate complete membrane reaction in >10 % of tumor cells (2+)
*SISH :
-Average HER2 copy No.:1.95
-HER2/CEP17 ratio:1.18
* Ki 67: 40 %

Diagnosis

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE.PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL EGYPTE

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Patient No.	: 29207272400329	Request Date	: 02/02/2026
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Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Left suspicious axillary lymph node , Tru cut needle biopsy
-Metastatic deposits of breast origin , focally necrotized.
IPT:
* ER: Positive (score 8/8).
* PR: Positive (score 8/8).
* HER2:Equivocal (score +2)
* SISH: Not Amplified
* Ki67: 40 %.
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells .IHC 0+:as defined as incomplete membrane reaction in less than 10 % of tumor cells . . IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.
ISH groups:
The ratio is calculated and the results is interpreted as:
• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE.PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

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Age	: 33 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result). Presidential Initiative for Women's Health- Pathology Lab Templates
• Group V (Negative): HER2/CEP17 ratio is less than 2

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
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Pathology Report
[[DOCTYPE.PATHOLOGY_REPORT]]

NILE SCAN & LABS

مركز النيل للأشعة والتحاليل

NILE DIAGNOSTIC CENTER

Name : Rabah Nadi Dahi Hassan	Date : 19-01-2026 09:39
Branch : Nasrcity	Medical No. : 295143
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

Whole Body (FDG) PET/CT Scan

Clinical question:

- 33-year-old lady with history of managed right breast cancer (IDC) underwent mastectomy in 20/10/2024 and on hormonal therapy.
- PET/CT is requested for status evaluation.

Procedure:

- Dose/Radiopharmaceutical: 4 mCi ^{18}F-FDG administered via a peripheral upper limb vein.
- Blood glucose level: 81 mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- CT without contrast images done for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the calf region.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
  The Standardised Uptake Value, maximum variant (SUV_{max}) was calculated within regions of interest whenever needed.

Findings:

The prior PET/CT DICOM images are not available for comparison.

Chest (including both breasts and relevant mammary LNs):

- **Evidence of right mastectomy with no definite FDG-avid focal lesions or masses at the operative bed.**
- Unremarkable appearance of the left breast.
- **FDG-avid enlarged left axillary lymph node, measuring ~ 3 cm with increased trace uptake eliciting SUV max ~ 11.9.**
- **Few enlarged metabolically active mediastinal right paratracheal, retrocaval, left internal mammary, subcarinal and right hilar LNs, reaching 1.5 cm and eliciting SUV max ~ 3.3.**
- **Few tracer starved sub-cm bilateral lung nodules.**
- No other enlarged or dense tracer fixing mammary related LNs.
- No lung masses or parenchymal infiltrates.
- No pleural or pericardial effusion.
- Normal tracer distribution over the myocardium.

PET Scan
[[DOCTYPE.PET_SCAN]]

NILE DIAGNOSTIC CENTER

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Head and neck:

- The brain looks normal with no abnormal focal metabolic activity. *(N.B: it may kindly be noted that some brain lesions not to be apparent on PET/CT and MRI is better for local assessment, if clinically warranted).*
- Normal FDG uptake pattern is seen at other soft tissues, glandular structures, oropharynx, larynx and salivary glands.
- No adenopathy or nodal hypermetabolism in the neck.

Abdomen and pelvis:

- Average-sized liver with **multiple FDG-avid hypodense hepatic focal lesions, reaching 1.4 cm and eliciting SUV max ~ 3.7.** No IHBR dilatation could be detected.
- No significant pathologically enlarged or metabolically active pelvi-abdominal lymph nodes.
- Normal spleen, pancreas, kidneys, suprarenals and pelvic structures.
- No ascites.

Musculoskeletal:

- **Multiple metabolically active mixed lytic/sclerotic osseous lesions, the most active (at DV12) eliciting SUV max ~ 9.4.**
- No other definite blastic or lytic bony lesions could be detected all over the surveyed skeleton.

Impression:

Whole body (FDG) PET/CT study is showing:

- **Status post right mastectomy with no definite FDG-avid focal lesions or masses at the operative bed that could account for active tumor biology.**

PET Scan
[[DOCTYPE.PET_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Rabah Nadi Dahi Hassan	Date : 19-01-2026 09:39
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Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

- FDG-avid nodal, HFLs and osseous deposits as cribbed, likely metastatic.
- Few bilateral tiny tracer starved lung nodules.
- No other FDG-avid focal lesions could be detected.
- Follow up FDG PET/CT is advisable as clinically warranted.

Best regards,

Dr. Mohammed Akl, M.Sc

MD

Dr. Alaa Nagi,

Nuclear Medicine

Dr. Mohamed Akl

Dr. Alaa Nagy

NILE DIAGNOSTIC CENTER

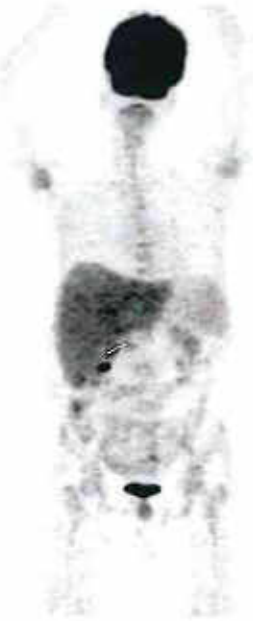
PET Scan
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Name : Rabah Nadi Dahi Hassan	**Date : 12-09-2026 07:49**
Branch : Maadi	**Medical No. : 295143**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : التامين الصحي**

Whole Body F18- FDG PET/CT study

Clinical history:- 34 year old female patient , presenting with right breast cancer underwent MRM received CTH , RTH and on hormonal referred for PET CT study
Type of the study :follow up 15/6/2026

rocedure:



- **Radiopharmaceutical:** 18F FDG, 3.2 mCi I.V.
- **Blood glucose level at time of 18F-FDG injection:** 87 mg/dL .
- **Time from 18F-FDG injection to scan:** 60 min
- **Technique :**MDCT diagnostic examination was taken for attenuation correction anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.
- **Patient's height: 150** cm **patient's weight:65** kg
- **SUV average of reference hepatic activity=1.4 .**
- **Measurements are correlated to body weight.**

PET/CT FINDINGS:-

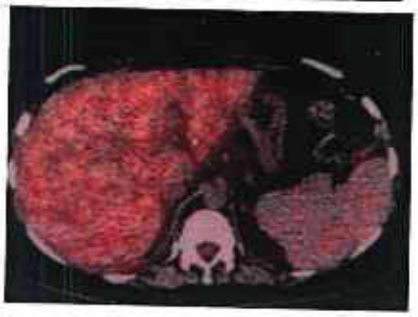
Head and Neck:

- No metabolically active cervical LNs noted.
- The brain exhibited normal FDG bio-distribution
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal thyroid uptake



Chest:

- Clear surgical right mastectomy bed with no FDG avid lesions seen
- Morphological regression with complete metabolic resolution as regard the previously noted metabolic active left axillary LN measures 17x15 mm
- Complete resolution of the metabolic active mediastinal LN, internal mammary and right supra clavicular LN
- Still noted bilateral non FDG avid pleural effusion mostly on the right side



NILE DIAGNOSTIC CENTER

Name : Rabah Nadi Dahi Hassan	**Date : 12-09-2026 07:49**
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Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : التامين الصحي**

- Normal FDG uptake is seen throughout the both lung lobes with no FDG avid lung nodules or masses detected.
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.

Abdomen and Pelvis:

- Stationary morphological course as regard the mottled liver appearance of multiple innumerable hypodense hepatic focal lesions the most avid seen at segment IV and V with SUV max of 7.7 (compared to SUV max of 10)
- No evidence of splenic FDG avid focal lesions.
- No metabolically active pelvi-abdominal LNs noted.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules
- Physiologic FDG excretion is seen in the kidneys and bladder.



Musculoskeletal:

- Still noted multiple mixed lytic and sclerotic bone lesions seen involving the axial and the appendicular skeleton reaching currently SUV max of 2.5 (compared to 5)

Conclusion:

- **Follow up PET/CT study reveled still clear right mastectomy bed with no FDG avid lesions to account for residual or recurrence**
- **Metabolic and morphological regression as regard the left axillary LN**
- **Complete resolution of the mediastinal , right supra clavicular , internal mammary LNs**
- **Stationary morphological course as regard the multiple osseous lesions with marked metabolic regression**
- **Stationary morphological course as regard the multiple hypodense hepatic focal lesions with regressive metabolic course**

Thanks for referral;
Dr. NORA NABIL , MD
Lecturer of Radio-diagnosis,
ASUH

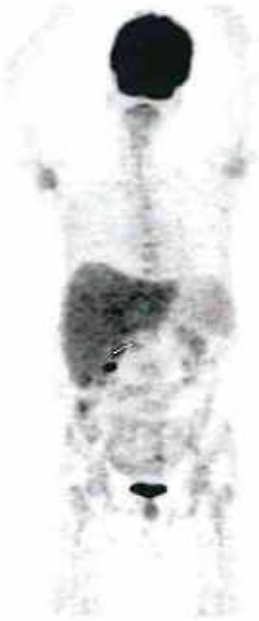
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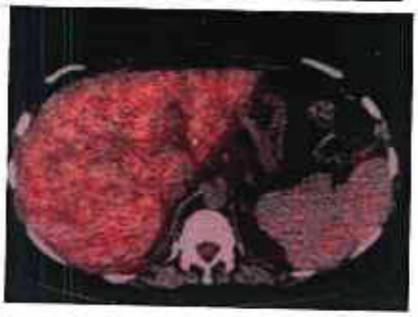
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- No metabolically active pelvi-abdominal LNs noted.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules
- Physiologic FDG excretion is seen in the kidneys and bladder.



Musculoskeletal:

- Still noted multiple mixed lytic and sclerotic bone lesions seen involving the axial and the appendicular skeleton reaching currently SUV max of 2.5 (compared to 5)

Conclusion:

- **Follow up PET/CT study reveled still clear right mastectomy bed with no FDG avid lesions to account for residual or recurrence**
- **Metabolic and morphological regression as regard the left axillary LN**
- **Complete resolution of the mediastinal , right supra clavicular , internal mammary LNs**
- **Stationary morphological course as regard the multiple osseous lesions with marked metabolic regression**
- **Stationary morphological course as regard the multiple hypodense hepatic focal lesions with regressive metabolic course**

Thanks for referral;
Dr. NORA NABIL , MD
Lecturer of Radio-diagnosis,
ASUH

NILE DIAGNOSTIC CENTER

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

DOCTOR'S ORDERS



Diagnosis: B.C Unit: Day care Diet Type: Normal diet

Date	Time Of Order	Doctor's Order	Physician Name & ID	Signature	Nurse Name & ID	Signature	Time of Order Execution
					Safa Nasser 3027	Safa Nasser	2:45 PM
27/8	12:40	[illegible]	[illegible]	[illegible]	[illegible]	[illegible]	2:30 PM

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.
* Do NOT use abbreviations.
*This form is NOT intended for use for medications orders.

الرقم الطبي:	6588
اسم المريض:	رياح نادى ضاحى -
تاريخ الميلاد:	27/07/1992
الرقم الطبي القديم:	null

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient name:	رياح نادى ضاحى
MRN:	1060/2026
	نفقة دولة نقدى

Blood Delivery form

	Components	Amount		Order Status	Time of Order
☐	Whole blood		☐	Scheduled	
☐	Packed RBCs	One Unit	☐	Emergency	
☐	Filtered Packed RBCs		☐	High Emergency (withou tcross matching)	
☐	Irradiated Packed RBCs		☐		
☐	Fresh Frozen Plasma		☐		
☐	Platelet concentrate		☐		
☐	Cryo precipitate		☐		
☐	Irradiated Platelet		☐		

Time of Delivery of Blood Components: 2:19:55 م

Blood Group: (O) RH Group (D antigen): Positive

Antibody Screening: ----------

Cross Matching: Matched

Bag No.:

	2610	A4969414
	Donation time	EXPIRATION DATE
	10/8/2026	14/9/2026

Donation time	EXPIRATION DATE	

Blood Bank Physician Name & ID	Signature	Date of Delivery	Time
محمود بشير	محمود بشير	27-2026-أغسطس	2:19 م

Assigned Nurse/ Healthcare Assistant Name & ID	Signature	Date of Delivery	Time
		27-2026-أغسطس	2:19 م

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

الرقم الطبي:	6588
اسم المريض:	رباح نادى صاحى .
تاريخ الميلاد:	27/07/1992
الرقم الطبي القديم:	null

Blood/ Blood Components Request

Room no:	Sex: ☐ Male ☐ Female	Age:
Diagnosis:		
Indication for transfusion:		
History of Previous Blood Transfusion:	☐ No ☐ Yes	
History of Previous Blood Transfusion Reaction:	☐ No ☐ Yes, specify.....	

Number of Units Needed:

- ☐ Whole Blood..... Units
- ☐ Packed Cells..... Units
- ☐ Platelets.....Units
- ☐ F.F.P..... Units
- ☐ Cryoprecipitate..... Units
- ☐ Auto transfusion..... Units
- ☐ Other: ..... Units

To be delivered to:

- ☐ OR
- ☐ ED
- ☐ ICU
- ☐ BMT

Urgency:

☐ **Immediate:** Needed in less than 20 minutes (Without compatibility testing).

☐ **Urgent:** Cross matching tests take **1 hour**

☐ **Routine:** compatibility test take two hours

Frequency of Transfusion:

Requesting Physician Name & ID:	Signature:	Date:	Time:
Sample taken by (nurse) Name & ID:	Signature:	Date:	Time

For Blood Bank:

Date	Blood Group	Bag no.	Time	Name & ID	Signature

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

الرقم الطبي:	6588
اسم المريض:	رباح هادى صاحى .
تاريخ الميلاد:	27/07/1992
الرقم الطبي القديم:	null

P:

Blood /Blood Components Transfusion Consent
اقرار بالموافقة على نقل الدم / مشتقاته

أقر انا الموقع ادناه (المريض/ ممثل القانوني)..... رقم قومي / جواز سفر.....

الرقم الطبي : ..... التشخيص: .....

أني قد وافقت على اعطاء الدم و/او احد مشتقاته و ذلك بعد ان قام الطبيب بشرح ما يلى بشكل واضح و مفهوم:

١- سبب الحاجة الى نقل الدم فى حالات:

أ- نقل خلايا الدم الحمراء: في حالة معاناة الشخص من فقر الدم الذي غالبًا ما يحدث بسبب نقص الحديد أو المعاناة من اضطراب الدم.

ب- نقل الصفائح الدموية: لتعويض المفقود لدى الأشخاص الذين يعانون من انخفاض في عدد الصفائح الدموية نتيجة العلاج الكيميائي أو اضطراب الصفائح الدموية

ج- نقل البلازما: تحتوي البلازما على بروتينات مهمة لصحة الجسم وغالبًا ما يتم نقلها للأشخاص في حالة المعاناة من الحروق الشديدة أو الالتهابات أو فشل الكبد.

٢- الفوائد المتوقعة من هذا الاجراء

٣- المخاطر و المضاعفات المحتملة بما فى ذلك:

أ- ردود الفعل التحسسيةو التى قد تشمل قد تشمل طفح جلدي، حكة، أو تورم.

ب- ارتفاع درجة الحرارة.

ج- العدوى النادرة بامراض فيروسية و بكتيرية.

د- مشاكل في القلب أو الرئة نتيجة فائض السوائل أو التحسس الشديد، تفاعل مع خطأ في المطابقة أعراق الدم، مما قد يؤدي إلى تفاعل جسدي شديد.

هـ- حدوث فرط الحديد: قد يتسبب فرط الحديد بتلف في مناطق وأعضاء مختلفة من الجسم إن لم يتم تداركه وعلاجه في الوقت المناسب، لا سيما إن حدث في القلب أو الكبد.

و- لإصابة بمشكلات صحية في الرئتين، مثل: إصابة الرئة المتعلقة بنقل الدم، وهو مرض نادر قد يتسبب بتلف الرئة والوفاة.

٤- البدائل الممكنة لنقل الدم ان وجدت.

٥- حقى فى طرح الاسئلة و الحصول على الاجابات الكاملة قبل اتخاذ القرار.

اقر انا المقع ادناه بانى على كامل المعرفة و الفهم لكل مشتملات هذا الاقرار و اتحمل المسؤلية كاملة عن كل ما فيه

اسم المريض (او ممثله القانونى): ..... اسم الطبيب المعالج/ الرقم الوظيفى: .....

درجة القرابة: ..... التوقيع: .....

رقم قومي / جواز سفر: ..... التاريخ/ الوقت: .....

التوقيع: : .....

اسم الشاهد: : .....

التاريخ/ الوقت: .....

رقم قومي / جواز سفر: .....

التوقيع: : .....

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient

الرقم الطبي:	6598
اسم المريض:	رباح هادي صاحب
تاريخ الميلاد:	27/07/1992
الرقم الطبي القديم:	null

Blood Transfusion Monitoring Form

Date of delivery: 27/8/26 Time of delivery: 12:45 Transfusion start time: 2.30pm End Time: 4:35

Blood Group & RH Type: O+ Type of blood unit: O+ Blood unit No: 26/0

Blood unit double checking:	☐ Assigned nurse	Name & ID: Ahmed mohamed 1703 Signature: Ahmed mohamed	☐ Charge Nurse	Name & ID: Sara Nasser 3027 Signature:
Patient Identification double checking (bedside):	☐ Assigned nurse	Name & ID: Ahmed mohamed 1703 Signature: Ahmed mohamed	☐ Charge Nurse	Name & ID: Sara Nasser 3027 Signature:

Nursing Observation: ☒ No reaction ☐ Reaction (Rash, Fever, Shivering, Facial edema, Laryngeal Spasm, ...)

If reaction, Notified to: .....Time: ... : ... Documented in Blood Transfusion Reaction Form ☐ Yes ☐ No

	Time	Temp.	Pulse	SPO2	RR	BP	Pain
Baseline vital signs	2:30	36-5	87	100	16	116/72	0
After 5 minutes	2:35	36-5	88	100	17	112/74	0
After 10 minutes	2:40	36-6	87	99	16	115/78	0
After 15 minutes	2:45	36-7	89	98	18	119/80	0
After 30 minutes	3	36-6	92	99	17	122/84	0
After 45 minutes	3:15	36-7	93	100	18	124/85	0
After 60 minutes	3:30	36-8	95	99	19	125/86	0
After 90 minutes	4	36-7	94	100	18	122/82	0
After completion of transfusion	4:35	36-8	96	99	17	126/84	0
After 5 minutes from completion of transfusion	4:40	36-9	95	98	16	122/81	0
After 1 hour from completion of transfusion	5:35	37	97	99	17	124/82	0
After 2 hours from completion of transfusion	6:35	36-8	98	100	18	126/83	0

Nurse Name & ID: Ahmed mohamed 1703 Signature: Ahmed mohamed Date: 27/8/2026 Time: 2:30pm

* Use separate form for each unit transfused.